

Abrazo Arrowhead Neonatal Acuity Tool

Infant Body Systems	Nurse Ratio 1:2 or 1:1* Intensive Care	Nurse Ratio 1:2-3 Intermediate Care	Nurse Ratio 1:3 Continuing Care	Nurse Ratio 1:3-4 Continuing Care
Weight/age	Small baby first 72 hours			
Respiratory	- HFOV, Ventilator, NIPPV, CPAP/BCPAP, High flow or Vapotherm >4 liters - Chest Tube(s) - iNO	- High flow or Vapotherm ≤4 liters - Extubated or off NIPPV/CPAP/BCPAP <24 hours	O ² NP up to 2L	RA (or home oxygen without titration parameters)
Cardiovascular	- UAC/PAL for BP monitoring - Vasoactive medications/ volume support - Cardiopulmonary resuscitation or acute pulmonary cardiac arrest	N/A	N/A	N/A
A/B/Ds	N/A	A/B/Ds requiring frequent intervention	A/B/Ds self-recovering/Brady watches	N/A
Nutrition	- TPN/Lipids and/or IV fluids - First 24 hours of trophic feeds	- NPO - Partial/Trophic feeds - TPN/Lipids and/or IV fluids (weaning)	Full feeds (PO/NG/OG/Ad Lib)	Full feeds (PO/NG/OG/Ad Lib)
Neuro	- Active seizures - IV sedation (NON NAS) - Therapeutic Hypothermia/HIE*	NAS <u>with</u> IV medication	NAS with PO medication	NAS without medication/ESC Scoring
Hyperbilirubinemia	- Multiple bili-lights and IV hydration and/or exchange transfusion* - IVIG therapy	Phototherapy with IV hydration	Phototherapy	Phototherapy
Transfusions	Multiple in 12 hours (Packed RBCs, platelets, cryo, FFP, etc.)	Transfusion (Packed RBCs or platelets)	N/A	N/A
Infection	Unstable sepsis	Stable sepsis with antibiotics	Suspected sepsis with antibiotic	N/A
Lines	N/A	Infusing IV, PICC, UVC/UAC	- IV saline lock or TKO - PICC w/ long term antibiotics in stable neonate	IV saline lock
Hematology/ other	- Life threatening congenital anomaly - DIC/pulmonary hemorrhage <24 hours - Bedside surgical procedure*	N/A	N/A	N/A
Nursing Care	Nursing intervention/assessment q 1-2 hours	Nursing intervention/assessment q 2-3 hours	Nursing intervention/assessment q3-4 hours	Nursing intervention/assessment q3-4 hours

*1:1 staffing in the NICU: Hemodynamically unstable due to oxygenation and perfusion insufficiencies requiring frequent nurse interventions every 15 minutes including, but not limited to, multiple invasive lines, multiple vasopressor titrations, and fluid and/or blood resuscitation. HFOV w/ iNO until stable, Therapeutic Hypothermia, Exchange blood transfusions for first four hours, and neonatal death for the first four hours. (policy: CLN.03.02) *Bedside surgical procedures to be 1:1 during procedure.*

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Nursing Levels of Care	NBN (N1)	Level 2 (N2)	Level 3 (N3)	Level 4 (N4)
Newborn Nursery	Newborn Nursery	Continuing Care Nursery (CCN), Special Care Nursery	Neonatal Intensive Care Unit (3)	Neonatal Intensive Care Unit (4)
These guidelines are meant to provide parameters for standardization. Multiple line items can increase the level of care depending on the nursing interventions required as well as state regulation criteria. Documentation is a critical component. Hyperbilirubinemia should resolve within 5 days. Subset is for 4 day LOS from hyperbilirubinemia identified/admitted Completed 7/15/24.				
Neurotoxicity Risk Factors: Isoimmune hemolytic disease, G6PD deficiency, Sepsis, Clinical Instability in last 24 hour, <38 weeks, albumen of 3 g/dL or less	X			
Current Birth Hospitalization and TSB rate of rise ≥ 0.3 mg/dL/hr and ≤ 24 hours from birth and phototherapy	X			
Current Birth Hospitalization and TSB rate of rise > 0.2 mg/dL/hr and > 24 hours from birth and phototherapy	X			
No neurotoxicity risk factors and TSB ≥ 5.5 to > 20 depending on hours from birth and gestational age ≥ 35 weeks and phototherapy	X			
Neurotoxicity risk factors and TSB ≥ 4 to > 16.5 depending on hours from birth and gestational age ≥ 35 weeks and phototherapy	X			
Current birth hospitalization and requires continued stay (days 2-5) with phototherapy, hyperbilirubinemia unresolved				
		X		
Readmission with failed home phototherapy		X		
No neurotoxicity risk factors and TSB ≥ 10 to > 21.5 depending on hours from birth and gestational age ≥ 35 weeks and Intensive Phototherapy (irradiance of more than 30uW/cm per nm)		X		
Neurotoxicity risk factors and TSB ≥ 6 to > 18.5 depending on hours from birth and gestational age ≥ 35 weeks and Intensive Phototherapy (irradiance of more than 30uW/cm per nm)		X		
Hyperbilirubinemia unresolved or worsening and Intensive Phototherapy (irradiance of more than 30uW/cm per nm)		X		
Continued Stay/Monitoring for post transfusion exchange complication < 48 hours				
			X	
No neurotoxicity risk factors and TSB ≥ 12 to > 25 depending on hours from birth and gestational age ≥ 35 weeks and Intensive Phototherapy and IV Hydration or Immunoglobulin			X	
Neurotoxicity risk factors and TSB ≥ 12 to > 25 depending on hours from birth and gestational age ≥ 35 weeks and Intensive Phototherapy and IV Hydration or Immunoglobulin			X	
Continued Stay (days 2-5) Hyperbilirubinemia unresolved or worsening and Immunoglobulin			X	
Continued Stay (days 2-5) Monitoring for TSB monitoring $\geq 4x$ in 24 hours and Intensive Phototherapy (irradiance of more than 30uW/cm per nm) and IV Hydration				X
				X
Hyperbilirubinemia and symptoms of encephalopathy				X
No neurotoxicity risk factors and TSB ≥ 15.5 to > 27 depending on hours from birth and gestational age ≥ 35 weeks and Exchange Transfusion				X
Neurotoxicity risk factors and TSB ≥ 14 to > 23.5 depending on hours from birth and gestational age ≥ 35 weeks and Exchange Transfusion				X
Continued Stay (days 2-5) with hyperbilirubinemia unresolved or worsening and exchange transfusion				

Nursing Levels of Care				
	NBN (N1)	Level 2 (N2)	Level 3 (N3)	Level 4 (N4)
Newborn Nursery	Continuing Care Nursery (CCN), Special Care Nursery	Neonatal Intensive Care Unit (3)	Neonatal Intensive Care Unit (4)	
These guidelines are meant to provide parameters for standardization. Multiple line items can increase the level of care depending on the nursing interventions required as well as state regulation criteria. Documentation is a critical component. Complete Oct 30, 2020 /Modified 5/24/21/ Modified 5/25/23 / Modified 7/15/2024				
Healthy infant >35 weeks gestation	X			
Babies who require routine evaluation and management of lab testing; glucose, bilirubin	X			
Babies who demonstrate prolonged transition after birth, requiring short term O2 therapy, monitored by pulse ox, who are able to be weaned to RA within the first 4-6 hours of life	X			
Babies who have NAS and need observation, but who are stable and do not require treatment	X			
Anti-infective and asymptomatic		X		
Babies being treated for seizure disorder that are stable		X		
Babies requiring continuous phototherapy after mother's discharge		X		
Babies requiring continuous servo controlled environment for thermoregulation		X		
Babies requiring evaluation and treatment for suspected sepsis		X		
Babies requiring gavage feeds		X		
Babies requiring supplemental respiratory support including short term CPAP or mechanical ventilation for less than 24 hours		X		
Babies requiring supplemental respiratory support of O2 therapy for <= 30 days FIO2 >21% to < 40% via hood, nasal cannula or <=1L/min HFNC		X		
Babies who demonstrate prolonged transition after birth, requiring short term O2 therapy, monitored by pulse ox, who are unable to be weaned to RA within the first 4 hours of life		X		
Babies with neonatal abstinence syndrome requiring treatment		X		
Feed and Grow		X		
Infants > 32 weeks or weigh > 1500 grams who do not have specific medical needs that meet level III criteria		X		
IV therapy, TPN once maintenance is established		X		
Short term central lines UAC, UVC, PICC (discuss with facility NICU medical director for parameters <3, 5, 7 days)		X		
Anti-infective and SYMPTOMATIC (hypotonia, tachypnea; tachycardia; temperature instability)			X	
Babies requiring arterial catheterization for frequent ABG analysis or monitoring (hemodynamically stable)			X	
Babies requiring red blood cell transfusion - blood product transfusion			X	
Babies requiring supplemental respiratory support of O2 therapy greater than or equal to 40%, NIPPV or mechanical ventilation or HFNC >1L/min			X	
Babies with acute cardiac instability, including acute CHF or unstable cardiac arrhythmia to include cardioversion			X	
Babies with birth asphyxia or 5 minute apgar of 6 or less			X	
Cerebral Spinal Fluid (CSF) drainage with documented hydrocephalus			X	
Complex Medical Condition - Stable			X	
External Ventricular drain (EVD - neurosurgery)			X	
Exchange transfusion, partial			X	
Infant requiring sustained life support			X	
Long term central lines UAC, UVC, PICC (discuss with facility NICU Medical Director for parameters > 3, 5, 7, 14 days)			X	
Peritoneal dialysis			X	
Surgical Repair of Stable Conditions			X	
TPN while adjusting towards maintenance, IVF Resuscitation, volume expanders				X
ECMO / ECLS				X
Exchange transfusion. Double volume (falls under hyperbilirubinemia - see tab below)				X
Hemodynamically unstable or highly likely to become hemodynamically unstable due to oxygenation and perfusion insufficiencies requiring frequent nurse interventions every 15 minutes including, but not limited to, multiple invasive lines, multiple vasopressor titrations, and fluid and/or blood resuscitation, Hemodynamic monitoring including arterial line, ICP Monitoring, Continuous EEG Monitoring				X
<= 72 hours				X
High Frequency Ventilation				X
Hemodialysis				X
Mechanical Ventilation and respiratory interventions every 1-2 hours (excluding weaning) FIO2> 50% or Oxygen Index > 15 or PEEP >8				X
Nitric Oxide				X
Prostaglandin for PDA Patency				X
Regional transport center unstable incoming or outgoing neonatal transports				X
Surgical repair of unstable complex conditions				X
Total Body Cooling				X